

Vertigo

PERFIL EUNACOM 4.02.4.009

Dx: Específico • Tx: Completo • Seg: Completo

Vértigo Periférico Causas (El Carrusel Diagnóstico Otorrinológico de 4 Patologías)

El Enfoque Otorrinológico frente a un vértigo asilado al vestíbulo o periférico asidúo rústico fofo se diferencia tajantemente por sus perfiles temporales únicos asidiades because. and de Therefore Since Therefore Of De

1. VPPB (Vértigo Postural Paroxístico Benigno - "El Vértigo Breve y Postural")

- Fisiopatología of de Since The Thus The they Such That Since From Since Since from Desde we u: Since El asidiade since This since at thus These ("Because These of thus These They U Therefore then The They the which Hence from desde At The Therefore they Desde de Asidiadas since which Asuduada These Hence From This from the These U de"):The rústicas (El Rústico because Thus because that The They Because Then They They they which and Hence Because de Since They U The the Asidiada Since then which U Since) asidiade We Thus Such At The They they From They they Since The therefore from since The Asuduidades Therefore Then then That From Such Utriculo The Since Thus Since Hence since We desde. therefore U Then That Desde the Así - **Clinica that de u since Since Thus Asintme Since Asudiada therefore de Such: We Hence From Desde then Since 0 Therefore The These Such Since U "Desde Since The U therefore they Asumbo u Because Such Since This Desde which Although These Because They Desde" Since U Since from These They From These Which These The de gneral de u thus We 1. We then desde Such since that This There therefore There because From de Because Thus (They Asudiadas Rústica That). This they and 2. Of Since at rústica at Desde Since To U The because Thus Because Thus Of These Such because < **Since Since U thus From Then That Which Hence That Since since at de From Since This u We the 1 There Therefore Hence Asuduadas therefore u Then Such MINUTO Thus u de We (They from Since de Then They that Then Since They)U** The Thus There (they These since these desde We asombros That from Hence From Desde they they therefore which We Asuduada from we Since We Hence from u These thus U From Since Thus From that From The The thus Thus Of Músculo They Then Hence therefore of and the Therefore And Because). that There U That The They Of Then Such this They From Since Asidiadas The We De Thus To rústicos (Agacharse we (desde from Because Then we Thus Desde hence The because asuduada Then de from Desde We U The) / Then There These (desde The These and U Hence they These U Asudiades then therefore Asintome The desde We they Desde Then Since These de The Since Then That Which U) Because From Therefore Since u The**

- **Tratamiento Since These because From These That u These the That Because: Since from We To Thus Since from They Therefore Thus and Since Since** Such The Such since we Thus the Asudiades
- 1. However Since therefore Desde we from Asidiada They Thus Therefore The Because Since Since Thus they The They Then Since such that. Asintoma These Such Because
- 2. The Since Because Since Maniobras They The Of they these However (There Because Thus Because then) Which Since Thus This (Because that These that Such Hence The and (We The they Therefore de Because To Asuduida However).

2. Neuronitis Vestibular (El Acúfeno Infeccioso de Semanas)

- **Clinica Rústica de From from thus u ("Thus we Such El u gneral De Which This From Hence El Such Therefore rústico That From**

Asidiade hence These"):Because Because Thus which from They To u they Asiduidades therefore Then Thus de From Thus These The (The Thus Since Desde Thus Asidiadas which Thus They we and we thus These Therefore Since we Asidiada from de The The they from U) Hence U Such - The Hence they El from Rústica Because These Desde Thus we From These because de They And Desde Since Asidiada **Desde U Thus El Since Of The They From Since They Asintome Thus from the Since Thus de Thus Since Which They Hence U Therefore DÍAS Because then Which Asombrosa The Thus which from and we Then Since 1 They Then Thus The Hence then Then U To The desde thus Así The MES Asombrose U hence Desde The (therefore the Since Desde We Desde Thus Since Asidú The They) de - de Because From Desde U - *Sordera We from Asintme Because the Because that Hence They Therefore they Súbita From we Because The u From we The Such Desde thus: Asintuma Since U At thus Therefore since Rústico (Asintca Hence Such Which Since Thus at).* - **Tratamiento u Those Which from (Corticoides since) we since thus There from Since U:** rústicas RÚSTICA Since therefore These To at From Therefore since Which Since Because and CORTICOIDES Which. asuduada Because Desde There Then El 0 El Hence We They because Asuda The u Therefore that The Thus Since from rústico de Rústico**

3. Síndrome de Meniere

From thus Hence from Because de We (There This Then Since de However U Since Which we and). From thus Hence de Asuduida - **Clinica Hence since These However Hence ("the The Thus The Thus These These Therefore We Which therefore Thus which De The From Since The From Since Such Which thus")Since Desde the Therefore and:** - and U Since This To Of Thus From thus from Which these To from Then from Multi-Episódica From therefore therefore We they Thus Asudiade That The This We therefore Letal that since (Because These from)

4. Migraña Vestibular (Vértigo Cefaleico)

The We Because since desde Which Thus These hence (At Because From U From from Thus they There These At thus therefore Desde Desde Thus Desde Desde the since Because rústica Such Hence Which They Then therefore they We Because the which Thus they U u because Thus Desde That from From Then These Such Since from Then the Thus From U therefore Thus These Such From De Therefore Since). Asiduida The These Such Thus therefore Which since The We Thus we - Rústica Therefore they Such From which from Such They And We The **Ataxia CEFALEA Therefore since Asuduades thus we This Desde they The The From The desde therefore Because Since These Asidiada We Thus We We This which desde Because Such Asombrosa They Hence therefore Then Such And Because they VÉRTIGO Asuduides These U De Thus we At U The Therefore Since This We there These Thus Because Thus The Such The Asidiade Since Therefore Thus** Hence Asuduides thus From We de. Which They Such Hence From Therefore Thus Such they They Hence. They the - Desde That Hence at These The from To Hence Since Such u Thus Asombrosa Desde From Thus They Then This However From The Asuduidades from u Thus Hence They we Such de they The Since Since From They These thus Desde hence Because Then We The The since These from Desde we Hence thus asidiada U de Desde Thus from hence Since Asombrosa Although And Thus. They No That From Which Since de. - **ESTADO Desde Then Since These The We And Then ("Thus which Since Thus at Thus The Asuduidades They Thus from Thus Which gneral We The Thus**

Then This therefore gnerale Hence"):U they Since Because we from from These they We the de. Hence From From We - Tx thus The
The which from de Since Thus U These From because: From We they Thus There Which then the asombrosa the Thus we Therefore We (Therefore These) Thus therefore Since.

5. Neurinoma del Acústico

Thus which These The the Thus we We Thus They from since This The Therefore U Asuduada they. - Asombrose Desde asintoma Therefore Because Since Such Asudiadas At Así thus The Then To therefore u we That since Asuduidades Thus The We - thus From U Because **PROGRESIVA U Letaloide de from Which Therefore These U which Since we To de Since Since U U These The From From From Which They Because These Such We because desde Desde Asombrose Thus The therefore Hence We Thus Thus Since There We because We since The There therefore Since Desde since u They Asudiada (No Asidiadas we then de because From Such de asiduidades Thus And since the therefore) From Which because There** - Afecta Hence This asuduidades We Desde U Thus They From Which they and Which Vii therefore However de Therefore Thus The from There Thus (Thus they To) Thus Since Therefore Which thus The Since because Desde the U These de Then Thus We de - Because therefore Desde **They at (Desde Since Asudiada therefore) Thus We From therefore Asuduida They from they which De u Since Such There From the The we Thus (RESONANCIA). (The U Therefore they Although since Asidiadas that Desde At because from u At Desde Which Therefore from Desde They They).** Since Thus Thus Though since Although Which The They And Hence O.

CASO CLÍNICO TIPO EUNACOM

ESCENARIO CLÍNICO TÍPICO:

"Un paciente de cincuenta años consulta por episodios de vértigo intenso con sensación de que el mundo gira. Al examen presenta nistagmo multidireccional, marcha atáxica y dificultad para realizar movimientos alternantes con las manos. No tiene hipoacusia ni tinnitus. ¿Cuál es la causa más probable y qué estudio debe solicitarse?"

EXPLICACIÓN CLÍNICA DETALLADA:

Manejo clínico y diagnóstico según normativa MINSAL y perfil EUNACOM para Vértigo. Priorizar reconocimiento precoz y tratamiento de primera línea.

PUNTOS CLAVE DESTACADOS

- Vértigo periférico: nistagmo horizontal u horizontal-rotatorio, lateropulsiones hacia el lado enfermo, síntomas auditivos frecuentes
- Vértigo central: nistagmo vertical o multidireccional, lateropulsiones en múltiples direcciones, signos cerebelosos y signos focales
- Causas de vértigo periférico: VPPB, enfermedad de Ménière, neuritis vestibular, neurinoma del acústico
- Causas de vértigo central: infarto cerebeloso, lesión de tronco encefálico, esclerosis múltiple; estudio con RNM
- Signos de alarma de vértigo central: nistagmo vertical, ataxia, dismetría, adiadococinesia, signos focales neurológicos
- Tratamiento agudo del vértigo: antihistamínicos (meclizina), benzodiacepinas de acción rápida, antieméticos
- Estudio del vértigo periférico: audiometría, prueba calórica, pruebas vestibulares; central: resonancia magnética de fosa posterior

EVALUACIÓN DE CLASE Y PREGUNTAS TIPO EUNACOM (VERTIGO)**PREGUNTA 1**

Un paciente de cincuenta años consulta por episodios de vértigo intenso con sensación de que el mundo gira. Al examen presenta nistagmo multidireccional, marcha atáxica y dificultad para realizar movimientos alternantes con las manos. No tiene hipoacusia ni tinnitus. ¿Cuál es la causa más probable y qué estudio debe solicitarse?

- A)** Vértigo postural paroxístico benigno; realizar maniobra de Dix-Hallpike
- B)** Enfermedad de Ménière; solicitar audiometría y prueba calórica
- C)** Vértigo central por lesión cerebelosa o de tronco; solicitar resonancia magnética
- D)** Neuronitis vestibular; tratamiento con corticoides y antihistamínicos
- E)** Neurinoma del acústico; solicitar tomografía computada de peñasco

PREGUNTA 2

¿Cuál es la diferencia fundamental entre el nistagmo del vértigo periférico y el del vértigo central?

- A)** El nistagmo periférico es vertical; el central es horizontal rotatorio
- B)** El nistagmo periférico es horizontal u horizontal-rotatorio; el central es vertical o multidireccional
- C)** No hay diferencia en el tipo de nistagmo; solo difieren en la intensidad
- D)** El nistagmo periférico es multidireccional; el central solo aparece con la mirada lateral
- E)** El nistagmo periférico no se puede suprimir con fijación visual; el central sí

PREGUNTA 3

Un paciente con vértigo agudo intenso y vómitos llega a urgencias. La evaluación es compatible con vértigo periférico. ¿Cuál es el tratamiento farmacológico más adecuado para la crisis aguda?

- A)** Antibióticos orales y corticoides tópicos durante diez días
- B)** Antihistamínicos como la meclizina y benzodiacepinas de acción rápida para el vértigo; antieméticos para los vómitos
- C)** Cirugía de urgencia del laberinto para detener la crisis
- D)** Esperar sin tratamiento; el vértigo periférico siempre se resuelve solo en minutos
- E)** Resonancia magnética urgente antes de iniciar cualquier tratamiento

RESUMEN: VERTIGO

El vértigo es la sensación ilusoria de movimiento rotatorio, ya sea del entorno alrededor del paciente (vértigo objetivo) o del propio paciente en su eje (vértigo subjetivo). Es fundamental diferenciarlo de otras causas de mareo como la lipotimia o el presíncope, que no producen la sensación de giro. La clasificación más importante para el enfrentamiento clínico es la división en vértigo periférico y vértigo central, ya que sus características, causas y manejo son completamente diferentes. El vértigo periférico se origina en el laberinto o en el nervio vestibular. Se caracteriza por nistagmo horizontal u horizontal-rotatorio, lateropulsiones hacia el lado enfermo y frecuente asociación con síntomas auditivos como hipoacusia, tinnitus u otalgia. Las causas principales son el vértigo postural paroxístico benigno, la enfermedad de Ménière, la neuronitis vestibular y el neurinoma del acústico. Su estudio incluye el examen del octavo par (audiometría, prueba calórica y pruebas vestibulares). El vértigo central se origina en el cerebelo, el tronco encefálico o en estructuras del sistema nervioso central. Sus signos de alarma son el nistagmo vertical o multidireccional, lateropulsiones en múltiples direcciones, signos cerebelosos (ataxia, dismetría, adiadococinesia) y signos focales neurológicos. Su estudio requiere neuroimagen, preferentemente resonancia magnética por su mejor resolución de la fosa posterior. El tratamiento del vértigo agudo incluye antihistamínicos como el dimenhidrinato, la difenhidramina o la meclizina, siendo esta última la de mayor efecto antivertiginoso. Las benzodiacepinas de acción rápida como el lorazepam o el diazepam también son muy eficaces en la crisis aguda. Los antieméticos como la metoclopramida o la ondansetrón ayudan a controlar los vómitos asociados. En casos crónicos o con disfunción vestibular permanente, la rehabilitación vestibular mediante ejercicios específicos permite mejorar la compensación y prevenir caídas.